

Patient Safety Incident Response Policy

Issue Date	Review Date	Version
March 2025	May 2029	V2

Purpose

This policy supports the requirements of the Patient Safety Incident Response Framework (PSIRF) and sets out University Hospitals Plymouth NHS Trust's (the Trust) approach to developing and maintaining effective systems and processes for responding to patient safety incidents and issues for the purpose of learning and improving patient safety.

The PSIRF advocates a co-ordinated and data-driven response to patient safety incidents. It embeds patient safety incident response within a wider system of improvement and prompts a significant cultural shift towards systematic patient safety management.

This policy supports development and maintenance of an effective patient safety system that integrates the four key aims of the PSIRF and which we can also align to our existing Trust values:

- compassionate engagement and involvement of those affected by patient safety incidents (*We put people first, we respect and value each other, we are compassionate, we listen, learn, and improve*)
- application of a range of system-based approaches to learning from patient safety incidents (*We take ownership, we listen, learn, and improve*)
- considered and proportionate responses to patient safety incidents and safety issues (*We take ownership, we listen, learn, and improve*)
- supportive oversight focused on strengthening response system functioning and improvement (*We put people first, we take ownership, we respect and value each other, we are compassionate, we listen, learn, and improve*).

This policy should be read in conjunction with the Trust's current Patient Safety Incident Response Plan (PSIRP), which is a separate document setting out how this policy will be implemented.

Who should read this document?

All staff

Key Messages

This policy supports the requirements of the Patient Safety Incident Response Framework (PSIRF) and sets out University Hospitals Plymouth NHS Trust's (the Trust) approach to developing and maintaining effective systems and processes for responding to patient safety incidents and events for the purpose of learning and improving patient safety.

Aims of PSIRF include the compassionate engagement and involvement of those affected by patient safety incidents. The application of a range of system-based approaches to our learning from patient safety incidents.

Core accountabilities

Owner	Head of Safety, Quality and Governance
Consultation	PSIRF Stakeholder Away Day May 2023 Stakeholder consultation October 2023 ICB presentation and sign off February 2024
Ratification	Chief Nurse
Dissemination (Raising Awareness)	Incident Team Manager
Compliance	Patient Safety Incident Response Group

Links to other policies and procedures

Patient Safety Incident Response Plan
Reporting of Injuries, Diseases and Dangerous Occurrences (RIDDOR) SOP
Duty of Candour and Being Open

Version History

V1.0	May 2024	Initial Policy
V1.1	December 2024	Links to other policies and procedures updated. Section 17: Health & Safety Team monitoring and oversight.
V2	February 2025	Policy updated: To reflect the patient safety incident definition and that incidents can be raised from other sources other than just via Datix reporting. I.e. patients via Complaints/PALs, staff via Freedom to Speak Up). Also to reflect changes to committee structure, amendments to Trust values, the different types of review responses available and simplified process flow chart.

The Trust is committed to creating a fully inclusive and accessible service. Making equality and diversity an integral part of the business will enable us to enhance the services we deliver and better meet the needs of patients and staff. We will treat people with dignity and respect, promote equality and diversity and eliminate all forms of discrimination, regardless of (but not limited to) age, disability, gender reassignment, race, religion or belief, sex, sexual orientation, marriage/civil partnership and pregnancy/maternity.

**An electronic version of this document is available on Trust Documents.
Larger text, Braille and Audio versions can be made available upon request.**

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1. Introduction

This policy is specific to patient safety incident responses which are conducted solely for the purpose of learning and improvement across the Trust.

Responses under this policy follow a systems-based approach. This recognises that patient safety is an emergent property of the healthcare system: that is, safety is provided by interactions between components and not from a single component. Responses do not take a 'person-focused' approach where the actions or inactions of people, or 'human error', are stated as the cause of an incident.

There is no remit to apportion blame or determine liability, preventability, or cause of death in a response conducted for the purpose of learning and improvement. Other processes, such as claims handling, human resources investigations into employment concerns, professional standards investigations, coronial inquests, and criminal investigations, exist for that purpose. The principle aims of each of these responses differ from those of a patient safety response and are outside the scope of this policy.

For clarity, the Trust considers these processes as separate from any patient safety investigation. Information from a patient safety response process can be shared with those leading other types of responses, but other processes should not influence the remit of a patient safety incident response.

2. Purpose

This policy supports the requirements of the Patient Safety Incident Response Framework (PSIRF) and sets out University Hospitals Plymouth NHS Trust's (the Trust) approach to developing and maintaining effective systems and processes for responding to patient safety incidents and issues for the purpose of learning and improving patient safety.

The PSIRF advocates a co-ordinated and data-driven response to patient safety incidents. It embeds patient safety incident response within a wider system of improvement and prompts a significant cultural shift towards systematic patient safety management.

This policy supports development and maintenance of an effective patient safety system that integrates the four key aims of the PSIRF and which we can also align to our Trust values:

- compassionate engagement and involvement of those affected by patient safety incidents (*We put people first, we respect and value each other, we are compassionate, we listen, learn, and improve*)
- application of a range of system-based approaches to learning from patient safety incidents (*We take ownership, we listen, learn, and improve*)
- considered and proportionate responses to patient safety incidents and safety issues (*We take ownership, we listen, learn, and improve*)

- supportive oversight focused on strengthening response system functioning and improvement (*We put people first, we take ownership, we respect and value each other, we are compassionate, we listen, learn, and improve*).

Incidents are reported to the Learn From Patient Safety Events Service (LFPSE), we will continue to review our incident management system to ensure new questions for LFPSE are updated and that the Taxonomy of LFPSE is updated. We will monitor LFPSE for externally reported incidents. We will review LFPSE outputs and reports.

This policy should be read in conjunction with the Trust's current Patient Safety Incident Response Plan (PSIRP), which is a separate document setting out how this policy will be implemented.

3. Definitions

Patient Safety Incident - Patient safety incidents are any unintended or unexpected incidents which could have, or did, lead to harm for one or more patients receiving healthcare. The event can be reported via multiple sources including: incident reporting to Datix, patient feedback via Complaints or PALS and staff via Freedom to Speak Up.

Patient Safety Incident Response Framework (PSIRF) – Replaces the Serious Incident Framework (2015) and is a contractual requirement under the NHS Standard Contract. PSIRF represents a significant shift in the way the NHS responds to patient safety incidents. The framework sets out the NHS's approach to developing and maintaining effective system and processes for responding to patient safety incidents for the purpose of learning and improving patient safety.

Patient Safety Incident Response Plan (PSIRP) – Our local plan sets out how UHP intends to respond to patient safety incidents over a period of 12 to 18 months. Our local safety priorities have been developed through a co-production approach with key stakeholders across the Trust supported by analysis of local data.

Restorative Just Culture - A culture where colleagues feel supported and empowered to learn when things do not go as expected, rather than allocating blame.

Learning Response Methods:

Review / Assurance Process	Purpose	Guidance notes
Immediate Safety Review (ISR) (nationally known as a swarm huddle)	Is designed to be initiated as soon as possible after an event It is a staff discussion about what happened and why it happened and identifying what can be done to reduce the chance of it happening again	• These should be completed by service lines on all reported incidents • Care Groups leads can provide advice and support upon request • Template available
Go Sees	Walk round of a specific area which includes discussion with staff and patients Can take the form of a 'fly on the wall' observation	• Ordinarily these should be undertaken without warning • Helps with understanding how 'work as done' is different to 'work as prescribed / imagined'. • There are blank observation templates available.
Patient Experience Survey (getting the patients voice)	To be used to provide information in deciding themes and terms of reference. For triangulation as part of quality pillars.	• Survey of patient experiences. This may either be through a specific national process (FFT or CQC survey) or locally driven process via the active listening scheme (overseen by the Corporate team) • Examples of surveys are readily available
Audit	Where specific assurances are required, an audit may be commissioned	• An audit provides a systematic review of care against explicit criteria with the outcome to either provide assurance or suggest improvement areas
Patient Safety Review (PSR) (nationally known as an After Action Review)	PSR is a structured facilitated discussion of an event, the outcome of which gives individuals involved in the event understanding of why the outcome differed from that expected and the learning to assist improvement PSR generates insight from the various perspectives of the MDT and is based on four questions: 1. What was the expected outcome/ expected to happen? 2. What was the actual outcome/ what actually happened? 3. What was the difference between the expected outcome and the event? 4. What have we learnt?	• These should be commissioned in response to specific incidents of concern and preceded by an Immediate Safety Review (see below) • The review is facilitated by Care Group leads and Learning Response Leads can provide advice and support upon request • Template available
Appreciative Inquiry (AI)	Appreciative Inquiries will be commissioned to understand key themes of concern, that are not included on the trust's Patient Safety Incident Response Plan Our Appreciative Inquiries involves x3 steps, with the intention of either providing assurance on the care delivered or identifying	• Appreciative Inquiries will be commissioned by the Patient Safety Incident Response Group but may be requested from any source

	improvement opportunities. The steps taken for a complete review include: 1. Data Pull - the data is based around the 7 pillars of governance, with additions based upon the specifics of the topic 2. Go See 3. Multidisciplinary Team Discussion – This happens through the Care Delivery Group	• Appreciative Inquiries will make recommendations to improve care processes (where necessary) and will only be undertaken by the Corporate Quality Governance Team
Care Group Investigation (nationally known as a mandated PSII)	To provide in-depth understanding in response to specific incidents of concern or where they meet mandated national requirements (Never Events, Death due to problems in care) Using the national PSII template These should be more in-depth than a Patient Safety Review	• To be commissioned through Care Groups on the advice of PSIRG. Terms of Reference for Mandated cases to be agreed at PSIRG and undertaken by Care Groups or Service Line leads • Where concerning themes are identified through the investigation, these should be escalated to the PSIRG for further consideration • Learning Response Leads can provide advice and support upon request • Template available
Learning Response (nationally known as a themed PSII)	Conducted to identify underlying system factors that contributed to an incident. These findings are then used to identify effective, sustainable improvements by combining learning across multiple Learning Responses and other responses into a similar incident type. Recommendations and improvement plans are then designed to effectively and sustainably address those system factors and help deliver safer care for our patients	• To provide in depth understanding of key areas as identified by the Trust's Patient Safety Incident Response Plan. • Learning responses will be commissioned into those areas highlighted in the plan. They will not be commissioned in response to specific incidents • Learning response shall only be commissioned through the Patient Safety Incident Response Group and shall only be undertaken by the Trust's Learning Response Leads • Template available
Mortality Review	A Mortality Review should be considered in response to any death in which previous concerns have been raised, or in response to a Mortality Trigger (e.g. a raised SHMI)	• Reviews should be heard through the Mortality Review Group primarily but may be requested in other governance forums • To review the key drivers of mortality in a specific service line or subject area. These will usually be commissioned through the Care Group or MRG (but PSIRG may recommend one) • The process of review will vary, but a Terms of Reference should be drafted and agreed through either Care Group or Corporate Structures prior to reviews commencing • Support can be provided by the Mortality and Clinical Harm Team or Head of Quality, Safety & Governance upon request

Structured Judgement Review	A Structured Judgement Review should be undertaken on any death that meets the following criteria: - Deaths of those with learning disabilities or severe mental illness - Deaths where the staff raise significant concerns about the care - Deaths where the bereaved raise significant concerns about the care - Deaths in a speciality, diagnosis or treatment group where an "alarm" has been raised (for example, an elevated mortality rate, concerns from audit, CQC concerns) - Deaths where learning will inform the provider's quality improvement work - Deaths where the patient was not expected to die - for example, in elective procedures - Maternal or neonatal deaths - Deaths where staff or bereaved raise significant concerns about the care - Care Group initiated SJR - Mortality Screening Tool Triggered SJR	- SJRs will ordinarily be commissioned through the Medical Examiner's Office or following the screening of a death by the Service Line. They should be completed by Service Line leads and outputs overseen by Care Groups - PSIRG or Mortality Review Group may also recommend a SJR - Defined structure
Child Death Review Meeting (CDRM)	This is a statutory process triggered upon notification of the death of a child and follows a national process. These are overseen by the Mortality and Clinical Harm team. Applies to all children up to the age of 18 who have died in the community and in hospital, but does not include: - Stillbirth: baby born without signs of life after 24 weeks gestation - Late foetal loss: where a pregnancy ends without signs of life before 24 weeks gestation	Outcomes of the CDRM may be requested to provide assurance
Perinatal Mortality Review Tool (PMRT)	This is a statutory process triggered upon notification of a neonatal death and follows a national process. These are overseen by the neonatal team	
Dashboards	To be used as a monitoring tool of a theme. Dashboards do not provide assurance themselves but should be used to identify areas that may need further review. To provide information in deciding themes and terms of reference. For triangulation as part of quality pillars	Dashboards should be held at Corporate, Care Group and Service Line level. Dashboard data should not be used to manage, but rather provides a "smoke signal" for where care may be deteriorating. Dashboard data should therefore contain high level metrics that provide an indication of care

4. Oversight Roles and Responsibilities

The Trust Board is responsible and accountable for effective patient safety incident management within the organisation. Whilst working under PSIRF, the Trust has designed its system for oversight

in a way that allows the organisation to demonstrate improvement, rather than compliance with prescriptive centrally mandated measures.

The Trust has followed the 'mindset' principles to underpin the processes we have put in place to allow us to implement PSIRF:

1. Improvement is the focus
2. Blame restricts insight
3. Learning from patient safety incidents is a proactive step towards improvement
4. Collaboration is key
5. Psychological safety allows learning to occur
6. Curiosity is powerful.

The Trust has identified the Chief Medical Officer and Chief Nursing Officer & Director of Integrated Professions as PSIRF Executive Leads who will support the responsibilities outlined below.

The Chief Medical Officer and Chief Nursing Officer & Director of Integrated Professions will oversee the development, review and approval of the organisation's policy and plan ensuring they meet the expectations set out in the patient safety incident response standards.

The Chief Nursing Officer & Director of Integrated Professions portfolio includes Quality Improvement, Clinical Effectiveness, Patient Safety, Patient Experience and Complaints.

The Chief Medical Officer and Chief Nursing Officer & Director of Integrated Professions will ensure PSIRF is central to overarching safety governance arrangements with clear mechanisms for the ongoing monitoring and review of the patient safety incident response plan, delivery of safe actions and improvements. The Trust Board will be provided with assurance that PSIRF and related workstreams have been implemented to the highest standards.

The Trust's Patient Safety Incident Response Group (PSIRG) via the Chief Medical Officer and Chief Nursing Officer & Director of Integrated Professions will ensure that Learning Responses and ISRs are conducted to the highest standards and to support the executive sign off process and ensure that learning is shared, and safety improvement work is adequately directed.

Updates will be made to this policy and associated plan as part of regular oversight. A review of this policy and associated plan will be undertaken at least every four years alongside a review of all safety actions.

5. Our Patient Safety Culture

All Reviews will be undertaken with a patient focus, with an emphasis on quality, whilst being cognisant of operational efficiency and pressures, and ensuring we build on a just and restorative culture.

PSIRF will enhance the Trust's transition to a restorative just culture by creating much stronger links between a patient safety incident and learning and improvement. We aim to work in collaboration with those affected by a patient safety incident – staff, patients, their families, and carers to arrive at such learning and improvement within the culture we hope to foster. This will continue to increase transparency and openness amongst our staff in reporting of incidents and engagement in establishing learning and improvements that follow. This will include insight from when things have gone well and where things have not gone as planned.

We are clear that patient safety incident reviews are conducted for the sole purpose of learning and identifying system improvements to reduce risk. Specifically, they are not to apportion blame, liability or define avoidability or cause of a patient's death.

University Hospitals Plymouth NHS Trust have developed a "Building Brilliance" programme, which considers the latest developments with regards to Patient Experience, Safety and Effectiveness through collaboration with staff and patients. The programme looks to assess the presence of the basic routines and processes that support the delivery of Safe, High-Quality care whilst also identifying areas of innovation and best practice that require sharing and celebrating. In this way, the programme will look to support a culture and environment that delivers Safe, High-Quality care in an improving environment.

The Building Brilliance programme takes the key components of peer review, innovation and understanding our routines and processes to introduce a programme that should support;

- Improvements in all areas of care quality, including Safety, Patient Experience and Effectiveness;
- A platform for shared learning, so that we can learn from each other and disseminate good practice;
- Increased staff engagement and team working with a subsequent impact on turnover, sickness, and consistency of staffing;
- A culture of pride and accomplishment, supporting reverse leadership and therefore personal and professional development;
- Setting of consistent expectations, providing clarity and direction for staff;
- Provision of consistent ward-to-board assurance on the quality of care.

The Building Brilliance programme looks to encourage and develop a culture where-by UHP staff undertake a process of self-review through appreciative enquiry, using this to identify learning and improvement opportunities, and in this way, it should help enhance our safety culture.

The Building Brilliance programme also looks to support the implementation and application of the "brilliant basics". These are the routines, processes and systems which act as the fundamental

foundations upon which an environment that supports safe, high-quality care is created and will therefore further raise our safety profile.

PSIRF processes and the Building Brilliance programme will be complementary to each other, as both will support a culture of innovation and learning whilst allowing for the identification of systematic weaknesses that impact the safety of care, we can provide our patients. The effectiveness of PSIRF processes and the Building Brilliance programme will be overseen and monitored via the Care Improvement Group.

6. Patient Safety Partners

The Patient Safety Partner (PSP) is a new and evolving role developed by NHS England to help improve patient safety across the NHS in the UK. A Patient Safety Partner (PSP) is actively involved in the design of safer healthcare at all levels of the organisation, ensuring that the patient voice is heard with the core purpose of improving safety and quality.

University Hospitals Plymouth NHS Trust Patient Safety Partners participate in safety governance activity. Our Patient Safety Partners will ensure that any committee/ group of which they are a member considers and prioritises the service user, patient, carer, and family perspective and champions a diversity of views. We may ask our Patient Safety Partners to:

- Participate in the investigation of patient safety events, assist in the implementation of patient safety improvement initiatives, and develop patient safety resources which will be underpinned by training and support in collaboration with the Risk & Incident team.
- Sit on relevant Committees to support compliance monitoring, suggest how safety issues should be addressed and provide appropriate challenge to ensure learning and change in the development and implementation of relevant strategy and policy.

Our Patient Safety Partners are supported in their honorary role by the Patient Safety Specialist for the Trust who will provide expectations and guidance for the role.

The PSP placements are on an honorary basis and will be reviewed after one year to ensure we keep the role aligned to the patient safety agenda as this develops.

In addition to the role that our Patient Safety Partners already plays within UHP, the new Clinical governance Framework, introduced in 2024, will ensure that patient experience and co-design is at the heart of clinical decision making and improvement. This approach is supported by both our Patient Council and the corporate Patient Services team.

7. Addressing Health Inequalities

The Trust recognises that the NHS has a core role to play in reducing inequalities in health by improving access to services and tailoring those services around the needs of the local population in an inclusive way.

The Trust as a public authority is committed to delivering on its statutory obligations under the Equality Act (2010) and will use data intelligently to assess for any disproportionate patient safety risk to patients from across the range of protected characteristics.

Whilst responding to patient safety events we will directly address if there are any particular features of an incident which indicate health inequalities may have contributed to harm or demonstrate a risk to a particular population group, including all protected characteristics. When constructing our safety actions in response to any incident we will consider inequalities, and this will be inbuilt into our documentation and governance processes with the completion of an Equality, Quality Impact Assessment (EQIA) where necessary

We will also address apparent health inequalities as part of our safety improvement work. In establishing our plan and policy we will work to identify variations that signify potential inequalities by using our population data and our patient safety data to ensure that this is considered as part of the development process for future iterations of our patient safety incident response plan and this policy. We consider this as an integral part of the future development process.

Engagement of patients, families and staff following a patient safety incident is critical to the review of patient safety incidents and their response. We will ensure that we use available tools such as easy read, translation and interpretation services and other methods as appropriate to meet the needs of those concerned and maximise their potential to be involved in our patient safety incident response.

8. Engaging and Involving Patients, Families and Staff following a Patient Safety Incident

PSIRF recognises that learning and improvement following a patient safety incident can only be achieved if supportive systems and processes are in place. PSIRF supports the development of an effective patient safety incident response system that prioritises compassionate engagement and involvement of those affected by patient safety incidents (including patients, families, and staff). This involves working with those affected by patient safety incidents to understand and answer any questions they have in relation to the incident and signpost them to support as required.

University Hospitals Plymouth NHS Trust are firmly committed to continuously improving the care and services we provide. As well as meeting our regulatory and professional requirements for Duty of Candour, we want to be open and transparent with our patients, families, and carers because it is the right thing to do. This is regardless of the level of harm caused by an incident. We want to

learn from any incident where care does not go as planned or expected by our patients, their families, or carers to prevent recurrence.

We recognise and acknowledge the significant impact patient safety incidents can have on patients, their families, and carers. Those affected by patient safety incidents, can expect staff liaising with them to:

- Provide a meaningful apology
- Provide an individualised approach
- Provide sensitive communication
- Treat them with respect and compassion
- Provide guidance and clarity
- Ensure those affected are 'heard'
- Provide a collaborative and open approach
- Accept subjectivity
- Strive for equity.

These principles are aligned with the obligations of Duty of Candour that organisations must uphold. This policy should be read in conjunction with the Duty of Candour and Being Open Policy.

The Trust's Patient and Family information booklet, adapted from research by the 'Learn Together' team will be provided to those affected by a patient safety incident that is subject to a Corporate team or Care Group led Learning Response. The booklet includes clear information about the purpose of a learning response, what to expect from the process, ways to be involved and how they can prepare for this involvement and details support resources available to them. This resource can be made available in both digital and physical formats.

The Trust recognises that there are other forms of support that those affected by a safety incident (patients, families, carers, and friends) can access. This is beyond the support provided by our team, who have been trained to guide patients, families and carers through any investigation or learning review. Where necessary, we will signpost affected individuals to this support which includes (but is not limited to):

National Guidance for NHS Trusts engaging with Bereaved Families	https://www.england.nhs.uk/wp-content/uploads/2018/08/learning-from-deaths-working-with-families-v2.pdf
Learning from Deaths – Information for Families This resource explains what happens after a bereavement (including when a death is referred to a Coroner) and how families and carers should comment on care received.	https://www.england.nhs.uk/publication/learning-from-deaths-information-for-families/

Child Death Support The two links offer support and practical guidance for those who have lost a child in infancy or at any age.	https://www.childbereavementuk.org/grieving-for-a-child-of-any-age https://www.lullabytrust.org.uk/bereavement-support/
Complaints Advocacy The NHS Complaints Advocacy Service can help navigate the NHS complaints system, attend meetings, and review information given during the complaint process.	https://www.voiceability.org/about-advocacy/types-of-advocacy/nhs-complaints-advocacy
Healthwatch Healthwatch are an independent statutory body who can provide information to help make a complaint, including sample letters.	https://www.healthwatch.co.uk/
Parliamentary and Health Service Ombudsman (PHSO) The PHSO makes the final decisions on complaints for patients, families and carers deemed not to have been resolved fairly by the NHS in England, government departments and other public organisations.	https://www.ombudsman.org.uk/

The Trust is always looking at ways to improve how we support those affected by patient safety incidents. Therefore, those affected by a patient safety incident as part of the investigation process will be participate in a feedback exercise to ensure we have the correct support mechanisms available for our patients and their families in accordance with their personal needs.

9. Patient Safety Incident Response Planning

PSIRF supports organisations to respond to incidents and safety issues in a way that maximises learning and improvement, rather than basing responses on arbitrary and subjective definitions of harm. Beyond nationally set requirements, organisations can explore patient safety incidents relevant to their context and the populations they serve rather than only those that meet a certain defined threshold.

The Trust will take a proportionate approach to its response to patient safety incidents to ensure that the focus is on maximising improvement. Our Patient Safety Incident Response Plan will detail how this has been achieved as well as how the Trust will meet both national and local focus for patient safety incident responses.

Resources and training to support patient safety incident response

The Trust has committed to ensuring that we fully embed PSIRF and meet its requirements. We have therefore used the NHS England patient safety response standards (2022) to frame the resources and training required to allow for this to happen.

The Trust will have in place governance arrangements to ensure that learning responses are not led by staff who were involved in the patient safety incident itself or by those who directly manage those staff and not undertaken by staff in isolation.

The Risk & Incident team will support learning responses wherever possible and can provide advice on cross-system and cross-Care Group/Service Line working where this is required.

Staff support: Those staff affected by patient safety incidents will be afforded the necessary managerial support and be given time to participate in learning responses. All Trust managers will work within our just and restorative culture principles and utilise other teams such as Occupational Health and Wellbeing to ensure that there is a dedicated staff resource to support such engagement and involvement. Care Groups and Service Lines will have processes in place to ensure that managers work within this framework to ensure psychological safety.

The Trust will utilise both internal and, where required, external subject matter experts with relevant knowledge and skills, where necessary, throughout the learning response process to provide expertise, advice, and proofreading.

Training

The Trust will design and implemented a bespoke patient safety training package that ensures all staff are aware of their responsibilities in reporting and responding to patient safety incidents and to comply with the NHS England Health Education England Patient Safety Training.

In addition, all staff, clinical and non-clinical are expected to undertake level one of the Patient Safety Syllabus training (Essentials for patient safety). All staff at AFC band 7 (or equivalent) or above will also need to undertake level two of the Patient Safety Syllabus Training (Access to Practice).

Learning Response Leads training

Any Corporate Trust Learning Response will be led by those who have had a minimum of two days formal training and skills development in learning from patient safety incidents and experience of patient safety response.

Learning Response Leads must have completed level one (essentials of patient safety) and level two (access to practice) of the national patient safety syllabus.

Learning Response Leads will undertake appropriate continuous professional development in incident response skills and knowledge, and network with other leads at least annually to build and maintain their expertise.

Learning Response Leads contribute to a minimum of two learning responses per year.

Learning Response Leads will support Care Group Learning Responses where requested.

Competencies for Learning Response Leads

All staff leading learning responses will be able to:

- Apply human factors and systems thinking principles to gather qualitative and quantitative information from a wide range of sources.
- Summarise and present complex information in a clear and logical manner and in report form.
- Manage conflicting information from different internal and external sources.
- Communicate highly complex matters and in difficult situations.

Engagement and involvement training

Engagement and involvement with those affected is led by those with at least six hours of training in involving those affected by patient safety incidents in the learning process.

Engagement Leads must have completed level one (essentials of patient safety) and level two (access to practice) of the national patient safety syllabus.

Engagement Leads will undertake continuous professional development in engagement and communication skills and knowledge, and network with other leads at least annually to build and maintain their expertise.

Engagement Leads will contribute to a minimum of two learning responses per year. Records for this will be maintained by the Corporate Risk & Incident team, supported by the Head of Quality, Safety & Governance.

Competencies and behaviours for Engagement Leads

All staff who are engagement leads will be able to:

- Communicate and engage with patients, families, staff, and external agencies in a positive and compassionate way.
- Listen and hear the distress of others in a measured and supportive way.
- Maintain clear records of information gathered and contact with those affected.
- Identify key risks and issues that may affect the involvement of patients, families, and staff.
- Recognise when those affected by patient safety incidents require onward signposting or referral to support services.

Oversight training (for Trust Board and Executive team)

All patient safety incident response oversight will be led/conducted by those with the one day training in oversight of learning from patient safety incidents.

Those with an oversight role on our Trust Board and Executive team must have completed level 1 (essentials of patient safety) and level 1 (essentials of patient safety for boards and senior leadership teams) of the national patient safety syllabus.

All those with an oversight role in relation to PSIRF will undertake continuous professional development in incident response skills and knowledge, and network with peers at least annually to build and maintain their expertise.

Competencies for individuals in oversight roles

All staff with oversight roles can:

- Be inquisitive with sensitivity (that is, know how and when to ask the right questions to gain insight about patient safety improvement).
- Apply human factors and system thinking principles.
- Obtain through conversations and assess both qualitative and quantitative information from a wide range of sources.
- Constructively challenge the strength and feasibility of safety actions to improve underlying system issues.
- Recognise when safety actions following a patient safety incident response do not take a system-based approach (e.g. inappropriate focus on revising policies without understanding 'work as done' or self-reflection instead of reviewing wider system influences).

- Summarise and present complex information in a clear and logical manner and in report form.

10. Our Patient Safety Incident Response Plan

Our plan sets out how the Trust intends to respond to patient safety incidents over a period of 12 to 18 months. The plan is not a permanent set of rules that cannot be changed. We will remain flexible and consider the specific circumstances in which each patient safety incident occurred and the needs of those affected, as well as the plan.

The Trust has undertaken a thorough review of available patient safety incident insight and have engaged with internal and external stakeholders to define its PSIRP.

Through our analysis of patient safety insights we have defined 10 local patient safety priorities with 5 identified as requiring a Learning Response. These are areas where patient safety risk is known to exist, and it is judged that a series of local targeted Learning Responses will deliver learning and improvement.

We will apply a systems-based approach to learning.

For any incident not meeting the Learning Response criteria, or any other incident to those listed above, we propose these will be managed at a local level with ongoing thematic analysis via our existing Trust assurance processes which may lead to new or supplement existing improvement work.

11. Reviewing Our Patient Safety Incident Response Policy and Plan

Our patient safety incident response plan is a 'living document' that will be appropriately amended and updated as we use it to respond to patient safety incidents. We will review the plan every 12 to 18 months to ensure our focus remains up to date; with ongoing improvement work our patient safety incident profile is likely to change. This will also provide an opportunity to re-engage with stakeholders to discuss and agree any changes made in the previous 12 to 18 months.

Updated plans will be published on our website, replacing the previous version.

A rigorous planning exercise will be undertaken every four years and more frequently if appropriate (as agreed with our Integrated Care Board (ICB), NHS Devon) to ensure efforts continue to be balanced between learning and improvement. This more in-depth review will include reviewing our response capacity, mapping our services, a wide review of organisational data (for example, Learning Response reports, improvement plans, complaints, claims, staff survey results, inequalities data, and reporting data) and wider stakeholder engagement.

12. Responding to Patient Safety Incidents

Patient safety incident reporting arrangements

All staff are responsible for reporting any potential or actual patient safety incident on the Trust's incident reporting system, Datix.

Service Lines/ Departments will have daily review mechanisms in place to ensure that patient safety incidents can be responded to proportionately and in a timely fashion. This should include consideration and prompting to teams where Duty of Candour applies. Most incidents will only require local review within the service, however for some, where it is felt that the opportunity for learning and improvement is significant, these should be escalated to Care Groups (see Patient safety incident response decision-making below). Reviews will focus on 'process' to understand outcomes, reviewing the 'work system' for each process.

Care Groups will highlight to the Risk & Incident team any incident which appears to meet the requirement for reporting externally. This allows the Trust to work in a transparent and collaborative way with our ICB, regional NHS teams and the CQC, notifying them if an incident meets the national criteria for a Learning Response or if supportive co-ordination of a cross system learning response is required.

The Risk & Incident team will act as liaison with external bodies and partner providers to ensure effective communication via a single point of contact for the Trust.

Patient safety incident response decision-making

The Trust has arrangements in place to allow it to meet the requirements for review of patient safety incidents under PSIRF. Some incidents will require a mandatory Learning Response, others will require review by the Trust's Patient Safety Incident Response Group (PSIRG).

All suspected Never Events will be taken to PSIRG for group discussion and review. Never Events will also be themed and considered against each other to look for shared improvements. All Never Events will be escalated up to Executive level. Any Never Event incident that involves a LocSSIPs/NaTSSIPs will be considered by the Trust lead with a review of the LocSSIPs.

PSIRF itself sets no further national rules or thresholds to determine what method of response should be used to support learning and improvement. The Trust has developed its own response mechanisms to balance the effort between learning through responding to incidents or exploring issues and improvement work. In the work to create our plan we have considered what our incident insight and engagement with key internal and external stakeholders has shown us about our patient

safety profile. We have used this intelligence to build our local priorities for Learning Responses and our toolkit for responding to other patient safety incidents.

We have established a process for our response to incidents which allows for a clear 'Ward to Board' set of mechanisms allowing for oversight of incident management and our PSIRF response.

Service Lines will have escalation arrangements in place for the monitoring of patient safety incidents. This includes escalation of incidents which appear to meet the need for further exploration after a rapid review (via Immediate Safety Review Huddles). Such escalations include incidents that meet criteria for inclusion into themed Learning Responses, nationally mandated Care Group Investigations (PSII) or other review types due to the potential for learning and improvement.

The weekly Patient Safety Incident Response Group (PSIRG) may identify incidents that represent an emerging theme or signify an unexpected level of risk and / or incidents with potential for learning and improvement which fall outside of the Trust's Patient Safety Incident Response Plan. In these instances, the PSIRG will identify an appropriate, proportionate response to understand key drivers behind the concerns and identify improvement priorities.

The Patient Safety Incident Response Group (PSIRG) has delegated responsibility and will use the Trust's Patient Safety Incident Response Plan to allow proactive allocation of patient safety incident resources which balances effort between learning through responding to incidents or exploring issues and improvement work. Key stakeholders (eg. safety experts, Patient Safety Partner, and Specialist Advisors) will be invited where necessary.

The Trust's Patient Safety Incident Response Group (PSIRG) has oversight of Learning Responses and mandated Care Group investigations. The outcomes of Patient Safety Reviews will go to Care Delivery Group and Care Improvement Groups to ensure that recommendations are founded on a systems-based approach.

The Care Delivery Group's purpose is to understand the processes and systems that lead to care delivery, through the commissioning of reviews, ensuring that best practice and risks are identified and appropriately escalated.

The Trust's Care Improvement Group (CIG) will oversee safety actions and improvement initiatives to ensure they are commissioned, implemented, and embedded. They will consider if existing safety improvement plans or the establishment of such plans where they are required. Improvement opportunities will be communicated to the CIG by outcomes from incident reviews commissioned through the Patient Safety Incident Response Group (PSIRG) or via deep-dive reviews from the Care Delivery Group (CDG).

Outputs from PSIRG, CDG and CIG will be presented to the Clinical Quality Review Group on a bi-weekly basis provided by the chairs of the groups. The Trust's Clinical Quality Review Group will have overall oversight of PSIRF processes and will challenge decision making to ensure that the Trust Management Board can be assured that the true intent of PSIRF is being implemented within the organisation and the Trust continues to meet the national patient safety response standards.

The Trust's Incident management process is outlined in Appendix A.

Responding to cross-system incidents/issues

The Risk & Incident Team will forward those incidents identified as presenting potential for significant learning and improvement for another provider directly to that organisation's safety team or equivalent.

The Trust will work with partner providers and the relevant ICBs to establish and maintain robust procedures to facilitate the free flow of information and minimise delays to joint working on cross-system incidents. The Risk & Incident team will act as the liaison point for such working and will have supportive operating procedures to ensure that this is effectively managed.

The Trust will defer to NHS Devon ICB for co-ordination where a cross-system incident is felt to be too complex to be managed as a single provider. We anticipate that the ICB will give support with identifying a suitable reviewer in such circumstances and will agree how the learning response will be led and managed.

Learning acquired from collaborative external reviews will be fed into the Care Delivery Group and Care Improvement Group where the implementation of actions will be commissioned and monitored to lead to sustainable change and improvement. The Trust's Patient Safety Specialists will share learning from completed learning responses as applicable through the System Safety Specialist forum.

Timeframes for learning responses

Timescales for Learning Responses

Where a Learning Response is indicated, the investigation must be started as soon as possible after the patient safety incident is identified and should ordinarily be completed within one to three months of their start date. No local Learning Response should take longer than six months.

The time frame for completion of a Learning Response will be agreed with those affected by the incident, as part of the setting of terms of reference, provided they are willing and able to be involved in that decision. A balance must be drawn between conducting a thorough Learning Response, the impact that extended timescales can have on those involved in the incident, and the risk that

delayed findings may adversely affect safety or require further checks to ensure they remain relevant.

In exceptional circumstances, a longer timeframe may be required for completion of the Learning Response. In this case, any extended timeframe should be agreed between the Trust and those affected.

Timescales for other review methods Reviews must be started as soon as possible after the patient safety incident is identified and should ordinarily be completed within 30 working days or as agreed at PSIRG

Safety action development and monitoring improvement

The Trust acknowledges that any form of patient safety learning will allow the circumstances of an incident or set of incidents to be understood, but that this is only the beginning. To reliably reduce risk, better safety actions are needed.

University Hospitals Plymouth NHS Trust has systems and processes in place to design, implement and monitor safety actions using an integrated approach to reduce risk and limit the potential for future harm. Initial findings will be discussed with Subject Matter Experts (SME's) who will generate safety actions in relation to each of those defined areas for improvement. Following this, the Trust will have measures to monitor any safety action and set out review steps.

Safety actions in response to a defined area for improvement depend on engagement with those working in the area who understand the factors and constraints involved. To achieve successful improvement safety action development they will be produced in a collaborative way with a flexible approach from areas and the support of the Care Delivery Group.

The Care Delivery Group will use the process for development of safety actions as outlined by NHS England in the Safety Action Development Guide (2022) as follows;

1. Agree areas for improvement – specify where improvement is needed, without defining solutions
2. Define the context – this will allow agreement on the approach to be taken to safety action development
3. Define safety actions to address areas for improvement – focused on the system and in collaboration with teams involved
4. Prioritise safety actions – to decide on testing for implementation
5. Define safety measures – to demonstrate whether the safety action is influencing what is intended as well as setting out responsibility for any resultant metrics
6. Write safety actions – following SMART principles, including details of responsible owner, measurement, and monitoring arrangements

7. Monitor and review – to ensure safety actions are impactful and sustainable.

Where safety actions take time to develop and implement, the Trust will record the areas for improvement in any safety event report but note that the safety actions will be developed as part of a wider safety improvement plan.

Safety Action Monitoring

The Care Improvement Group (CIG) will ensure that improvement initiatives are identified, commissioned, and implemented across the organisation. Improvement opportunities will be communicated to the Care Improvement Group to oversee and receive assurance of progress with improvement plans.

Safety actions must continue to be monitored within Service Line and Care Group governance arrangements to ensure that any actions put in place remain impactful and sustainable. Care Group reporting on the progress with safety actions including the outcomes of any measurements will be made to the Care Improvement Group.

For some safety actions with wider significance, this may require oversight by the Quality Governance department or Improvement project Leads to the Care Improvement Group.

Safety improvement plans

Safety improvement plans bring together findings from various responses to patient safety incidents and issues. The Trust has several overarching safety improvement plans in place which are adapted to respond to the outcomes of improvement efforts and other external influences such as national safety improvement programmes or CQUINs.

The Trust's Patient Safety Incident Response Plan has outlined the local priorities for focus of investigation under PSIRF. These were developed due to the opportunity they offer for learning and improvement across areas where there is no existing plan or where improvement efforts have not been accompanied by reduction in apparent risk of harm.

The Trust will use the outcomes from existing patient safety incident reviews (Serious Incident Root Cause Analysis reports) where present and any relevant learning responses conducted under PSIRF to create related safety improvement plans to help to focus our improvement work.

Where overarching systems issues are identified by learning responses outside of the Trust local priorities, a safety improvement plan will be developed. Monitoring of progress with regard to safety improvement plans will be overseen by reporting by the designated lead to the Care Improvement Group on a scheduled basis.

13. Complaints and Appeals

University Hospitals Plymouth NHS Trust recognise that there will be occasions when patients, service users or carers are dissatisfied with aspects of the care and services provided by the Trust.

It is important to understand that there is a distinction made between complaints and concerns as the use of the word complaint should not automatically mean that someone expressing a concern enters the complaints process.

The first point of contact with the Trust is the Patient Advice and Liaison Service (PALS) who will support the resolution of any concerns raised. How to contact the PALS team:

- **PALS Hub:** The PALS team are available Monday to Friday 9.30am to 12.30pm and 1pm to 4.30pm in the PALS Hub on the main concourse of Level 6.
- **Telephone:** 01752 439884 (internally 39884) between 9.30am to 12.30pm and 1pm to 4pm
- **E-mail:** plh-tr.PALS@nhs.net
- **Post:** Patient Advice & Liaison Service, Patient Services Office, Level 7, Derriford Hospital, Plymouth, PL6 8DH
- **Out of Hours:** Out of hours there is the facility to leave a message for a member of the team to make contact, to use this facility please telephone 01752 439694. If you call out-of-hours a member of the team will return your call as soon as practicably possible.

Where individuals remain dissatisfied, or wish to make a formal complaint, they can write to:

Patient Services, Level 7, Derriford Hospital, PL6 8DH.

Telephone: 01752 439884 or email: plh-tr.complaints-patientservices@nhs.net.

We aim to resolve complaints as quickly as possible without the need for lengthy correspondence. We take all complaints seriously and investigate them thoroughly. Staff at University Hospitals Plymouth NHS Trust understand the importance of listening to what you have to say about our services. We aim to learn from our mistakes. Making a complaint will not have a negative effect on future care.

If help is needed writing a complaint, contact the Advocacy People (Independent Health Complaints Advocacy Service) on 0330 440 9000 or email at: info@theadvocacypeople.org.uk alternatively in writing at the following address: Plymouth Advocacy, Highbury House, 207 Outland Road, Plymouth PL2 3PF.

The Patient Advice and Liaison team can advise and help you if you if your postcode requires as different advocacy service.

14. Overall Responsibility for the Document

The overall responsibility for this document sits with the Risk and Incident Team, with oversight provided by the Patient Safety Incident Response Group.

15. Consultation and Ratification

The design and process of review and revision of this policy will comply with The Development and Management of Formal Documents.

The review period for this document is set as default of five years from the date it was last ratified, or earlier if developments within or external to the Trust indicate the need for a significant revision to the procedures described.

This document will be reviewed by the Patient Safety Incident Response Group and ratified by the Chief Nurse.

Non-significant amendments to this document may be made, under delegated authority from the Chief Nurse, by the nominated owner. These must be ratified by the Patient Safety Incident Response Group.

Significant reviews and revisions to this document will include a consultation with named groups, or grades across the Trust. For non-significant amendments, informal consultation will be restricted to named groups, or grades who are directly affected by the proposed changes.

16. Dissemination and Implementation

Following approval and ratification, this policy will be published in the Trust's formal documents library and all staff will be notified through the Trust's normal notification process.

Document control arrangements will be in accordance with The Development and Management of Formal Documents.

The document owner will be responsible for agreeing the training requirements associated with the newly ratified document with the named Director and for working with the Trust's training function, if required, to arrange for the required training to be delivered.

17. Monitoring Compliance and Effectiveness

For patient related incidents, compliance with this Policy will be monitored by the Risk & Incident Team. The output of this monitoring will be reported to the Patient Safety Incident Response Group (PSIRG) together with recommendations for improvement and actions to address any issues

arising. The PSIRG acts as the coordinating committee, seeking assurance on the effectiveness of the Trust's healthcare governance arrangements. PSIRG will seek assurance as part of the Quality Governance compliance assessment that all patient safety events are addressed appropriately.

The Health & Safety Team will review and monitor all reported health & safety incidents relating to staff, visitors, contractors, volunteers, as well non-clinical health & safety incidents relating to patients.

The Health & Safety team will report their findings to include any subsequent requirements to report specific incidents and injuries via RIDDOR to the Health & Safety Executive , to the Health & Safety Assurance Sub Committee, Health and Safety Partnership Sub- Committee, and for Executive oversight through the Safety Quality People & Culture Committee and the Annual Health & Safety Board Report.

18. References and Associated Documentation

Duty of Candour further guidance:

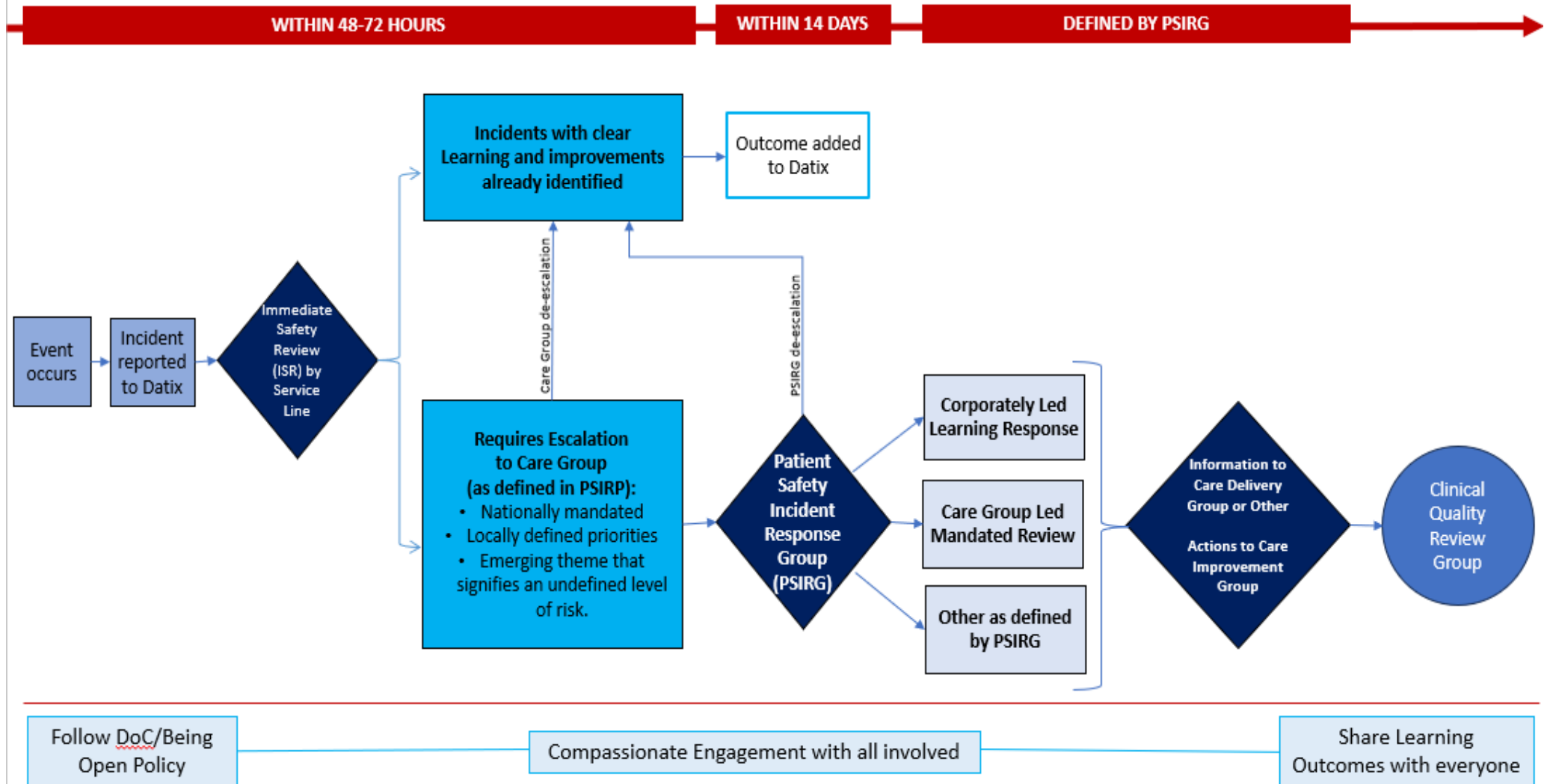
- CQC Handbook – Health and Social Care Act 2008 (Regulated Activities) Regulations 2014: Regulation 20.

<https://www.cqc.org.uk/news/stories/updated-guidance-meeting-duty-candour>

Appendix 1 - Incident Response System Process

UHP PSIRF PROCESS MAP

V2 February 2025



Appendix 2 - Dissemination Plan and Review Checklist

Dissemination Plan			
Document Title	Patient Safety Incident Response Policy		
Date Finalised	30/06/2024		
Previous Documents			
Action to retrieve old copies	N/A		
Dissemination Plan			
Recipient(s)	When	How	Responsibility
All Trust staff	June 2024	Information Governance StaffNet Page	Information Governance Team

Review Checklist		
Title	Is the title clear and unambiguous?	Y
	Is it clear whether the document is a policy, procedure, protocol, framework, APN or SOP?	Y
	Does the style & format comply?	Y
Rationale	Are reasons for development of the document stated?	Y
Development Process	Is the method described in brief?	Y
	Are people involved in the development identified?	Y
	Has a reasonable attempt has been made to ensure relevant expertise has been used?	Y
	Is there evidence of consultation with stakeholders and users?	Y
Content	Is the objective of the document clear?	Y
	Is the target population clear and unambiguous?	Y
	Are the intended outcomes described?	Y
	Are the statements clear and unambiguous?	Y
Evidence Base	Is the type of evidence to support the document identified explicitly?	Y
	Are key references cited and in full?	Y
	Are supporting documents referenced?	Y
Approval	Does the document identify which committee/group will review it?	Y
	If appropriate have the joint Human Resources/staff side committee (or equivalent) approved the document?	Y
	Does the document identify which Executive Director will ratify it?	Y
Dissemination & Implementation	Is there an outline/plan to identify how this will be done?	Y
	Does the plan include the necessary training/support to ensure compliance?	Y
Document Control	Does the document identify where it will be held?	Y
	Have archiving arrangements for superseded documents been addressed?	N/A
Monitoring Compliance & Effectiveness	Are there measurable standards or KPIs to support the monitoring of compliance with and effectiveness of the document?	Y
	Is there a plan to review or audit compliance with the document?	Y
Review Date	Is the review date identified?	Y
	Is the frequency of review identified? If so is it acceptable?	Y
Overall Responsibility	Is it clear who will be responsible for co-ordinating the dissemination, implementation and review of the document?	Y

Appendix 3 - Equalities and Human Rights Impact Assessment

Core Information	
Date	02/07/2024
Title	Patient Safety Incident Response Policy
What are the aims, objectives & projected outcomes?	The introduction of the new framework for developing and maintaining effective systems and processes for responding to patient safety incidents for the purpose of learning and improving patient safety.
Scope of the assessment	
The Equality Impact Assessment (EIA) has been undertaken to ensure that the publication of this policy is compliant with the Equalities Act 2010.	
Collecting data	
Race	N/a
Religion	N/a
Disability	N/a
Sex	N/a
Gender Identity	N/a
Sexual Orientation	N/a
Age	N/a
Socio-Economic	N/a
Human Rights	N/a
What are the overall trends/patterns in the above data?	None
Specific issues and data gaps that may need to be addressed through consultation or further research	None

Involving and consulting stakeholders				
Internal involvement and consultation	A range of key stakeholders (including internal Care Groups, Corporate teams, Patient Safety Partner, Specialist Advisors and Quality & Safety team colleagues from NHS Devon ICB.			
External involvement and consultation	NHS Devon ICB			
Impact Assessment				
Overall assessment and analysis of the evidence	This policy does not have a direct impact of equality and diversity however the range of new systems used to respond to a patient safety events may impact on staff on patients. This will be reviewed on an individual basis.			
Action Plan				
Action	Owner	Risks	Completion Date	Progress update